

AI Backend

Choose AI model:

- ☐ Offline Demo
- ☒ Gemini (free)

Gemini API Key

.....

Key set

Policy Documents

Document source:

- ☐ Sample CMS Policies
- ☒ Upload your own

Supports .txt and .pdf files

Healthcare Policy Content Analyzer

AI-powered policy comparison, rule extraction & provider alert generation
| Cotiviti Intern Assessment -- Likitha KB

Analysis completed in 189.6 seconds

Key Changes ...	HIGH Risks	MEDIUM Risks	Denial-Risk R...	Documents C...
31	6	1	34/37	2 P...
	↑ action r...	↑ monitor		

Comparing: 1.pdf vs 2.pdf

⚠ This is a provider communication draft generated from uploaded policy content. Any sentence prefixed with [AI Recommendation] is not explicitly sourced from the policy document. Review before sending.

Updates to Evaluation and Management (E/M) Documentation and Auditing Requirements

Effective: See policy update

We are updating Evaluation and Management (E/M) guidelines to align with our contractual obligation to enrollees. Key changes include replacing overall organ system counts with a detailed bulleted-element counting system for physical exams, requiring a Chief Complaint (CC) at all history levels, and establishing specific criteria for Comprehensive, Detailed, and specialty exams.

To comply, providers must:

- Align examinations with precise bullet-counting structures and shaded/unshaded box criteria.
- Ensure a CC is indicated at all history levels.
- Document the date, location, and updates when referencing historical ROS or PFSH.
- Elaborate on abnormal findings in symptomatic areas, reviews of old records, and family histories.
- Meet specific criteria for Comprehensive Musculoskeletal (all 4 elements across 4 areas) and Skin (all 4 areas) exams.

Failure to comply will result in claims denials (High Risk):

- Failing to document all shaded-border and at least one unshaded-border elements in Comprehensive exams.
- Documenting <12 elements for Detailed Neurological/Respiratory exams or <9 for Detailed Psychiatric exams.
- Omitting elaboration on old records, family history, or "abnormal" symptomatic findings.
- Failing to individually document 10 systems for a complete ROS without a "negative" notation.
- [AI Recommendation] Update your electronic health record templates immediately to prompt for the mandatory Chief Complaint at all history levels and enforce bulleted-element counting for Detailed and Comprehensive exams.

(Medium Risk: Reviews of historical ROS/PFSH omitting date, location, or updates will fail history requirements.)

Download as .txt

Download as .md

Email Preview

From: Provider Relations
<providerrelations@examplehealthplan.com>

provider-networks@example.com

To: In-Network Provider Network

Subject: Updates to Evaluation and Management (E/M)
Documentation and Auditing Requirements

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Neurological/Respiratory exams or <8 for Detailed

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Psychiatric exams.

- Omitting elaboration on old records, family history, or "abnormal" symptomatic findings.
- Failing to individually document 10 systems for a complete ROS without a "negative" notation.
- [AI Recommendation] Update your electronic health record templates immediately to prompt for the mandatory Chief Complaint at all history levels and enforce bulleted-element counting for Detailed and Comprehensive exams. (Medium Risk: Reviews of historical ROS/PFSH omitting date, location, or updates will fail history requirements.)